

# CARDIO-RESPIRATORY ARREST ADULT (signs of puberty are visible)

Unresponsive  
Open the airway (chin lift / jaw thrust):  
Check, listen, feel (max.10 sec.)  
Gasping is not considered efficient breathing  
Don't waste time checking pulse  
Evidence of trauma: consider maintain head-neck-trunk axis until final immobilisation

**CPR 30:2**  
- 30 compressions  
Push hard and fast in the middle of the chest  
5-6 cm but not more as 6 cm depth (2 hands)  
Rate of 100 (5 cycles) to 120 (6 cycles) per min.  
Allow for full chest recoil after each compression  
Minimise interruption in chest compressions  
- 2 ventilations with BVM+Oxygen in max. 10 sec.  
Visualisation of normal thorax extension  
If possible, use oropharyngeal cannula  
Manual suction device ready and available

Do not touch the patient during analysis

Do not touch the patient during analysis and shock  
After resuming CPR, check for signs of life[1]

Switch roles for compressions / ventilations

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Switch roles for compressions / ventilations

Do not touch the patient during shock  
After resuming CPR, check for signs of life[1]

Evacuation from the place of intervention to the ambulance: find the best balance between minimal interruption of chest compressions and moving to the ambulance in order to quickly reach the appropriate care center for advanced care

Patient evaluation

No sign of life

CPR 30:2 until AED is ready

Rhythm analysis

Shock indicated

Shock not indicated

1 shock CPR 30:2  
2 min. / 5-6 cycles

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Shock indicated

Shock not indicated

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Rhythm analysis

CPR 30:2  
2 min. / 5-6 cycles

After resuming CPR, check for signs of life[1]

Shock indicated

Shock not indicated

[1]Sign of life?

YES

Adequate ventilation

YES

NO

1 shock CPR 30:2  
Prepare for evacuation

CPR 30:2  
Prepare for evacuation

Evacuation

\* According to the SARC Manual Training & Oxygen Administration Protocol

CPR during transport

Transport to an appropriate health center (cardiology)

Upper Airway management

NO

Maintain SpO2 between 94-98%  
In any case, nonrebreather mask 10-15 l/min.\*  
Continue to monitor closely (possible new cardiac arrest)  
Keep temperature between 32-36°C

Evacuation

1 min. or less

1 min. or less

2 min.

2 min.

Anticipation of evacuation

Continuous assessment

The quality of the compression is a priority (Class I, LoE B).  
After 2 min. compressions-ventilations, replaces the person doing chest compressions (ERC, 2015)  
Limit ventilation volume, observe normal lifting of the chest (not to large volume, risk of filling the stomach and causing silent vomiting).  
Follow the SARC instructions in case of a deceased person.  
When possible, allow the family to see that the first responders are doing whatever is possible to save the patient's life.